Multiplier-Free Seizure Detection with a LUT-Native Neural Network on Behind-the-Ear EEG, ECG, EMG and Motion

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***Abstract*—Wearable seizure detection has to run for days on a small battery, which favours classifiers without multipliers. We study LUT-native neural networks, whose neurons are FPGA lookup tables, for patient-independent seizure detection from two-channel behind-the-ear EEG, ECG, EMG and motion in the SeizeIT2 dataset (125 patients, 11,626 hours, 883 seizures). A bit-exact integer front end computes 117 features without multipliers; a thermometer encoder and a Differentiable Weightless Neural Network (DWN) of 50 six-input LUTs turn them into a popcount score. On five-fold cross-validation over the 96 training patients the DWN reaches an event-F1 of 0.211 with 2.4 false alarms per day at about 960 estimated LUTs and no multipliers, seven to eight times fewer LUTs than TreeLUT and a logic-gate network, and better than an INT8 network that needs 9,600 multiplications per decision, though below gradient-boosted trees (0.292). Training with random sensor dropout keeps the detector working when the ECG is missing (event-F1 0.172 against 0.092, $p = 0.005$). On the 29 held-out test patients, [test result: SzCORE event metrics, mean ± std over 5 seeds, against clean ChronoNet and LightGBM]. We also report an independent reproduction of the published ChronoNet baseline, whose released code selects its model on the test patients.**

I. Introduction

Patients are often unaware of their own seizures, so diaries undercount them [1], and wearable detection is clinically recommended today only for tonic-clonic seizures [2]. Focal seizures that do not become tonic-clonic (822 of the 883 in SeizeIT2) remain the open problem, with false alarms the main obstacle [3]. Behind-the-ear (bte) EEG is discreet but sees only two channels; adding ECG, EMG and motion helps [4], [5], but the detectors that fuse them are floating-point software or multiply-accumulate networks on microcontrollers [6].

LUT-native networks replace weighted sums by lookup tables: each neuron is a Boolean function of a few input bits that maps one-to-one onto an FPGA LUT. DWNs [7], logic-gate networks [8] and LUT networks [9] train such networks by gradient descent, and tree ensembles can be mapped to LUTs in the same spirit [10]. They have been applied to ECG arrhythmia [11] and EEG classification [12], but not to seizure detection, which brings extreme class imbalance (0.12 % seizure time), patient-to-patient variability, missing sensors and event-level clinical scoring.

Contributions. (i) The first differentiable LUT-native seizure detector and the first benchmark of LUT-native models (DWN, logic-gate network, TreeLUT) against standard machine learning on the official SeizeIT2 split with SzCORE event scoring [13]. (ii) A bit-exact, multiplier-free integer front end for bte-EEG, ECG, EMG and motion, with a fixed-point specification and a reference Verilog classifier that matches the software on all test vectors. (iii) Modality dropout for a LUT network, which makes it robust to missing ECG and motion, selected among seven families of extensions by per-patient tests. (iv) An independent reproduction of the SeizeIT2 ChronoNet baseline, with a protocol that uses no test information.

II. Related Work

Wearable bte-EEG. Two bte channels capture many focal seizures [14]; ECG raises sensitivity by 8–11 % [4]. Tree ensembles won the ICASSP 2023 wearable-EEG challenge [15], [16]. The SeizeIT2 paper reports SVM and ChronoNet [17] baselines [18]. *Hardware detectors.* ASICs reach nJ–µJ per classification with SVMs and trees [19]–[21]; an SNN on a small iCE40 FPGA detects seizures on four scalp channels [22]; transformers and fusion networks run on microcontrollers [6], [23]. Almost all use EEG only, patient-specific training and epoch-level metrics. *LUT-native networks.* Beyond the works above, the thermometer encoder can dominate the cost of small DWNs [24], and a recurrent logic-gate network exists for language [25]. The only weightless seizure work is a WiSARD study on four patients [26].

III. Dataset and Protocol

Data. SeizeIT2 (OpenNeuro ds005873 v1.1.0) [18]: 125 patients with focal epilepsy from five European monitoring units, 2,850 recordings, 11,626 hours of two-channel bte-EEG, single-lead ECG and EMG at 256 Hz and accelerometer and gyroscope at 25 Hz; ECG/EMG are missing in 3.1 % and motion in 3.9 % of the time. Our audit found 883 seizures (702 in the 96 training patients, 181 in the 29 test patients; the paper states 886), and 256 Hz rather than the documented 250 Hz. Seizures were annotated on video and full-scalp EEG. *Split and tuning.* Official split: training sub-001–096, test sub-097–125. Every choice (features, hyperparameters, thresholds, post-processing, extensions) is made by five-fold patient-wise cross-validation inside the training patients; each final model is then trained on all 96 and scored once per seed on the test patients. *Scoring.* SzCORE event scoring [13] (any overlap, 30 s pre- and 60 s post-ictal tolerance, events closer than 90 s merged), averaged over patients, with false alarms per 24 h. A

decision is made every second; an alarm is raised when at least k of the last 10 decisions are positive; the threshold, k and an artefact gate are chosen on pooled out-of-fold predictions. Per-patient paired Wilcoxon tests and patient-bootstrap 95 % intervals; an extension is kept only if $p < 0.05$ in its favour.

IV. Method

A. Integer multimodal front end

Samples arrive as int16 (0.25 µV per count). Every filter is a second-order section with coefficients rounded to at most five signed powers of two and second-order error feedback; without it the rounding error of the 0.5 Hz high-pass exceeds the EEG itself. Every second, 2-s windows give per-channel and cross-channel EEG magnitude, line length, mobility, zero crossings, extrema and four band magnitudes; ECG heart rate and RR variability in the log domain (no division); EMG magnitude, line length, zero crossings and a band ratio; accelerometer and gyroscope magnitude, jerk, activity and band magnitude; per-modality presence bits; and, for most features, the difference from a causal 64-s moving average. On 10 training patients the integer features match a floating-point twin (median correlation 0.9965; LightGBM AUROC 0.736 against 0.735).

B. Thermometer encoding and DWN

Each of the 40 selected features is compared against 8 thresholds at training quantiles, $b_{f,i} = [x_f > t_{f,i}]$. A layer of 50 LUT6 neurons reads these bits through a learnable mapping and is trained with extended finite differences [7]; the score is the popcount of the seizure group minus that of the background group. Features are ranked inside each training set by the gain of a LightGBM model; this replaced mutual information after an interpretability study showed that mutual information ignored the heart-rate features (event-F1 0.148 $\rightarrow$ 0.208, $p = 0.047$). Estimated cost: about 850 LUTs of comparators, 50 network LUTs, 57 popcount and 12 post-processing LUTs.

C. Extensions and modality dropout

We tested seven families of extensions against this base: a stateful network with 16 state bits fed back each second and stacked windows (N1), encoder designs (N2), label-free per-patient threshold adaptation (N3), distillation from LightGBM and teacher-guided hard negatives (N4), focal, noise-robust and multiple-instance losses and onset weighting (N5), modality dropout, reliability-gated and late fusion (N6), and alarm interpretation (N7). The one adopted is *modality dropout*: during training, ECG, EMG and motion are each removed independently with probability 0.2 per window, exactly as when the sensor is absent (features and presence bits set to zero).

V. Results

A. Baseline reproduction

On the 29 test patients, our PyTorch port of ChronoNet with the released protocol detects 74.5 % of seizures at 22.6 false alarms per hour (published: 84.2 % at 100.5) with AUROC 0.741 (0.792); the per-recording AUPR (0.109) matches the published 0.104. The released code selects the training epoch on the test patients; a clean run (last epoch) gives 67.1 % at 11.9 false alarms per hour, and under SzCORE an event-F1 of 0.027 at 106 false alarms per day. The clean run is our ChronoNet reference.

TABLE I
Five-fold cross-validation on the 96 training patients (SzCORE, mean over patients; best-F1 operating point; hardware cost estimated).

Model	AUROC	F1	Sens	FA/24h	Cost
LightGBM	0.870	0.292	0.327	3.6	≈30.6k LUTs
CatBoost	0.863	0.286	0.337	3.0	≈27.7k LUTs
XGBoost	0.867	0.277	0.286	3.4	≈47.9k LUTs
TreeLUT (5-bit leaves)	0.857	0.266	0.329	4.2	≈6.4k LUTs
DWN, 50 LUT6	0.840	0.211	0.315	2.4	≈960 LUTs
DWN + modality dropout	0.849	0.188	0.275	1.9	≈945 LUTs
Random forest	0.852	0.198	0.216	0.5	≈323k LUTs
Logic-gate network	0.843	0.171	0.330	4.9	≈7.5k LUTs
INT8 MLP	0.830	0.147	0.225	7.8	9,600 mult.
Logistic regression	0.806	0.105	0.216	8.8	117 mult.

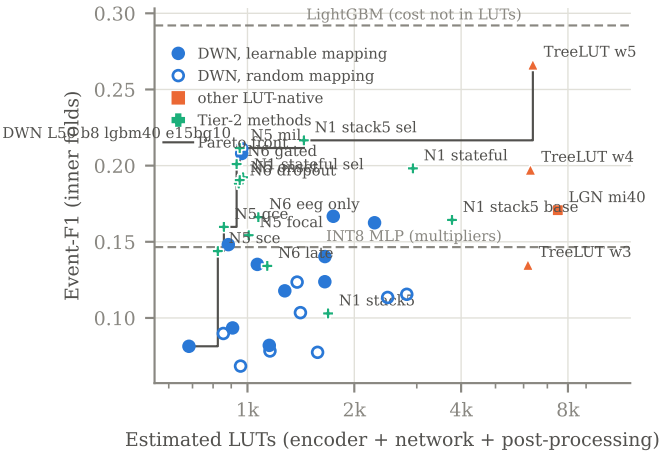


Fig. 1. Event-F1 against estimated LUTs on the training folds, with the Pareto front.

B. Model comparison (training patients)

Table I compares the model families on cross-validation. The DWN is the most compact model, with one of the lowest false-alarm rates, and beats the INT8 MLP on every metric; it remains below gradient-boosted trees (per-patient event-F1 against LightGBM, $p = 0.004$). Between 50 and 200 LUTs, size did not change event-F1 significantly; four thresholds per feature were worst at every size.

C. Extensions and missing sensors

None of the N1, N3 and N5 extensions improved event-F1 (Table **??**); the stateful network and onset weighting cut false alarms ($p = 0.013$, $p = 0.014$) at lower sensitivity, and the focal and noise-robust losses were significantly worse. The base loses most of its accuracy without ECG or motion (Table III). Modality dropout keeps it working: without ECG

TABLE II
TIER-2 METHODS AGAINST THEIR OWN BASELINES (INNER FOLDS; MEAN PER-PATIENT DIFFERENCE, BOOTSTRAP 95% INTERVAL, PAIRED WILCOXON).

Method	ΔF1 [95% CI]	p	ΔFP/24h	p	ΔLUTs
n1_stack5_sel	+0.006 [-0.041, +0.049]	0.25	-0.3	0.81	+478
n1_stateful_sel	-0.019 [-0.066, +0.026]	0.66	-1.3	0.013	+8
n5_focal	-0.057 [-0.106, -0.012]	0.038	-0.9	0.028	+44
n5_gce	-0.051 [-0.086, -0.019]	0.0052	-0.8	0.27	-106
n5_mil	+0.000 [-0.038, +0.037]	0.91	-0.6	0.11	-13
n5_onset	-0.021 [-0.058, +0.016]	0.28	-1.1	0.014	-13
n5_sce	-0.067 [-0.111, -0.027]	0.0031	-0.8	0.6	-139
n6_dropout	-0.023 [-0.066, +0.019]	0.23	-0.5	0.44	-19
n6_eeg_only	-0.045 [-0.096, +0.003]	0.084	+1.2	0.0014	+109
n6_gated	-0.010 [-0.062, +0.038]	0.92	-0.9	0.11	-32
n6_late	-0.077 [-0.125, -0.027]	0.00063	-1.9	4.1e-09	+173

TABLE III
EVENT-F1 WITH SENSORS REMOVED AT TEST TIME (TRAINING FOLDS, EACH MODEL AT ITS OWN OPERATING POINT).

Model	intact	no ECG	no EMG	no motion	EEG only
DWN base	0.211	0.092	0.199	0.136	0.074
+ modality dropout	0.188	0.172	0.190	0.192	0.112
late fusion	0.134	0.070	0.012	0.056	0.000

its event-F1 is 0.172 against 0.092 for the base ($p = 0.005$), without motion 0.192 against 0.136 ($p = 0.021$), at no significant cost when every sensor is present ($p = 0.23$). Late fusion, one small DWN per modality, collapsed whenever a modality was missing. [test result: N2 encoder and N4 distillation results, still running at the time of writing.]

D. Seizure types

Out of fold, the base detects 29 of 33 focal to bilateral tonic-clonic seizures (88 %), but 17 % of focal impaired awareness, 9 % of focal aware and 8 % of focal seizures with unknown awareness.

E. Held-out test patients

[test result: Table: SzCORE and paper metrics on the 29 test patients for the final DWN (modality dropout), LightGBM, TreeLUT, INT8 MLP, logistic regression and clean ChronoNet, mean ± std over 5 seeds, with per-patient tests against ChronoNet and LightGBM; results by seizure type and with sensors removed.]

VI. HARDWARE

The final DWN is exported as thresholds, input mapping, truth tables and popcount head, with test vectors for every stage from raw samples to alarms; a generated reference Verilog classifier matches the software on all test vectors in Icarus Verilog. [Phase 2: post-synthesis LUT, flip-flop and timing results and power on a low-power FPGA, with the INT8 MLP and TreeLUT through the same flow.] Until then, costs are analytical estimates.

VII. DISCUSSION AND LIMITATIONS

Accuracy per LUT favours the DWN by a wide margin, and the thermometer encoder, not the network, dominates its cost, as also measured by [24]. Focal seizures remain mostly undetected, consistent with the clinical evidence that wearable detection is established only for tonic-clonic seizures [2]. Results come from retrospective recordings of one dataset; the public release has no centre or age labels, so no centre or age breakdown is possible; and hardware costs are estimates until synthesis.

VIII. CONCLUSION

A 50-LUT weightless network on integer multimodal features detects seizures across patients with no multipliers and about 960 estimated LUTs, and modality dropout makes it robust to missing ECG and motion. [test result: one sentence on the test-patient result.]

REPRODUCIBILITY

Code, configurations, decision log, export packages and the Verilog generator are released with the paper. Dataset: SeizeIT2, DOI 10.18112/openneuro.ds005873.v1.1.0.

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